

Therapist Scope of Practice

Document ID	TG-POL-01	Effective	March 1, 2026
Applies to	Licensed Therapists (LCPC, LCSW, LMFT, PsyD, PhD)	Owner	Clinical & Compliance Team

At a Glance

In scope	Relationship distress, individual anxiety/depression (mild–moderate), grief, life transitions, attachment work, premarital counseling, trauma within trained modality.
Requires coordination	Stable bipolar disorder, personality disorder presentations without safety risk, moderate–severe anxiety/depression, borderline substance concerns.
Out of scope	Active psychosis/mania, substance use disorders needing detox, medically unstable eating disorders, forensic/custody evaluations, prescribing medication.
Immediate escalation	Any active suicidal or homicidal ideation with intent or plan — see Emergency Protocol (TG-POL-05).

1. Purpose & Applicability

This policy defines the clinical scope of practice for licensed therapists providing individual and couples sessions on the Togethr platform. It applies to all therapists engaged as employees or contracted providers, regardless of the state in which they are licensed, and is intended to be read alongside the Referral Protocol (TG-POL-04), Emergency Protocol (TG-POL-05), and Professional Conduct Standards (TG-POL-06).

Togethr is headquartered in Chicago, Illinois, and the majority of its clinical operations are governed by Illinois licensing law. Therapists licensed in other states remain bound by the licensing requirements of their home state and any applicable interstate telehealth compacts, in addition to this policy.

2. Licensure Requirements

All therapists must hold an active, unrestricted clinical license in good standing — Licensed Clinical Professional Counselor (LCPC), Licensed Clinical Social Worker (LCSW), Licensed Marriage and Family Therapist (LMFT), or a doctoral-level license (PsyD, PhD) with an active state credential. Provisional or associate-level licenses do not meet the bar for the Therapist role and should instead be reviewed against the Counselor Scope of Practice (TG-POL-02).

- Therapists must maintain active malpractice coverage as a condition of continued credentialing on the platform.
- License verification is performed at onboarding and re-verified annually through the Illinois Department of Financial and Professional Regulation (IDFPR) or the equivalent board in the therapist's home state.
- Therapists must self-report any change in license status (suspension, probation, complaint) to the Clinical & Compliance Team within 5 business days.

3. Conditions and Issues Within Scope

Therapists on Togethr are credentialed to address the following, consistent with their individual license and demonstrated competency areas on file:

- Relationship and marital distress, including communication breakdown, trust repair, and conflict patterns
- Individual anxiety and depressive symptoms of mild to moderate severity
- Grief and loss, including relationship-related loss (breakup, divorce, miscarriage)
- Life transitions (career change, relocation, new parenthood, retirement)
- Attachment patterns and their effect on adult relationships
- Premarital and pre-commitment counseling
- Family-of-origin issues as they present in current relationship functioning
- Trauma processing within the therapist's demonstrated competency and modality training (e.g., EMDR-certified therapists may treat trauma; others should refer)

4. Conditions Requiring Coordinated Care

The following may be treated by a therapist, but only with active coordination alongside a psychiatrist or other medical provider, and must be flagged in the client's file:

- Bipolar disorder that is currently stable on medication (acute mania or hypomania requires immediate referral — see Section 5)
- Personality disorder presentations that do not include active self-harm or safety risk
- Anxiety or depressive disorders of moderate-to-severe intensity where medication may be indicated
- Substance use that does not meet criteria for a use disorder but is a stated concern of the client or partner

Coordinated care must be documented in the client's file, including the name of the co-treating provider and the cadence of communication (with client consent on file).

5. Out of Scope — Mandatory Referral or Decline

Therapists must not attempt to treat, and must refer out per TG-POL-04, the following:

- Active psychosis or manic episodes
- Substance use disorders requiring medical detoxification or inpatient level of care
- Eating disorders with medical instability (must be co-managed with a specialist eating disorder team per TG-POL-04)
- Any presentation of active suicidal or homicidal ideation with intent or plan — this triggers Emergency Protocol (TG-POL-05), not a standard referral
- Forensic evaluations, custody evaluations, or any court-ordered assessment — Togethr therapists do not provide expert testimony or legal opinions
- Prescribing, adjusting, or advising on psychiatric medication — this is outside every therapist's license regardless of specialty

6. Session Structure

Individual sessions are 45–53 minutes; couples sessions are 60–80 minutes, consistent with the durations reflected in the platform's booking system. Sessions should not run over by more than 5 minutes except in an active safety situation, in which case Emergency Protocol applies.

7. Documentation Requirements

Session notes must be completed in the platform's clinical note template within 24 hours of the session (48 hours maximum for couples sessions with two note sets). Any safety-relevant disclosure, however minor, must be documented verbatim where possible and flagged using the in-app risk indicator.

Notes should include: presenting concern for the session, interventions used, client's affect and engagement, any homework or between-session tasks assigned, and plan for the next session. Notes are part of the clinical record and may be reviewed by a clinical supervisor or, in the event of a complaint, by the Clinical & Compliance Team.

8. Example Scenarios

The following illustrate how this policy applies in practice:

- A couple presents with ongoing arguments about finances and division of household labor. This is squarely within scope — relationship conflict and communication patterns.
- An individual client discloses a childhood history of trauma that resurfaces during a discussion about intimacy. If the therapist is trained in trauma modalities (e.g., EMDR, somatic approaches), this remains in scope; if not, a referral to a trauma-specialized colleague is appropriate.
- A client mentions they've been drinking more heavily since a recent breakup but does not meet criteria for a use disorder. This can be addressed directly, with monitoring, per Section 4.

- A client describes racing thoughts, decreased need for sleep, and impulsive spending consistent with a manic episode. This is outside scope and requires same-day referral per TG-POL-04; if there is any safety concern, Emergency Protocol (TG-POL-05) applies instead.

9. Frequently Asked Questions

Q: Can a therapist see both members of a couple individually as well as together?

A: Yes, with clear documentation of the treatment structure and consent from both parties regarding how information will (and will not) be shared between individual and joint sessions.

Q: Can a therapist continue seeing a client who has started psychiatric medication?

A: Yes — this is an example of coordinated care under Section 4. The therapist should document the coordination and, with client consent, communicate periodically with the prescriber.

Q: What if a client's condition seems borderline between in-scope and out-of-scope?

A: Consult your clinical supervisor before the next session. It is always acceptable to pause and ask; this is not treated as a performance concern.

10. Related Policies

This document should be read alongside: Referral Protocol (TG-POL-04), Emergency Protocol (TG-POL-05), and Professional Conduct Standards (TG-POL-06). For questions not answered by any of these, contact the Clinical & Compliance Team directly.